

UNITED STATES EDITION · SEPTEMBER 2026

USA Healthcare Market Report 2026

CMS National Health Expenditure, FDA, PBM and IRA intelligence for the world's largest healthcare market — pharmaceuticals, medical devices, Medicare negotiation and hospital purchasing.

COVERAGE

United States

HEADLINE NHE

USD 6.0T (CMS 2026)

USE

Launch & payer planning

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This PDF is the **USA Healthcare Market Report 2026** briefing aligned to bionixus.com/usa-healthcare-market-report and [/healthcare-market-research/united-states](https://bionixus.com/healthcare-market-research/united-states). National health spending uses **CMS Office of the Actuary NHE projections 2025–34 (released June 2026)**. Pharmaceutical and device bands are BioNexus published ranges, not a syndicated SKU feed. Not legal, medical or investment advice.

WHAT THIS REPORT IS — AND IS NOT

It is a board briefing: official CMS macro sizing, regulators, access steps, epidemiology and named hospital systems. Ranges that are BioNexus-owned are labelled as such. Therapy SKU shares, rebate guarantees and hospital-level consumption remain in commissioned work.

Live pages: bionixus.com/usa-healthcare-market-report · [/healthcare-market-research/united-states](https://bionixus.com/healthcare-market-research/united-states) · Related: [/canada-healthcare-market-report](https://bionixus.com/canada-healthcare-market-report) · [/brazil-healthcare-market-report](https://bionixus.com/brazil-healthcare-market-report)

02 United States at a glance

340M POPULATION (PLANNING YEAR)	\$6.0T NHE 2026 (CMS)	~\$615B PHARMA MARKET 2026	~\$180B DEVICES MARKET 2026
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The United States is the world’s largest healthcare market and accounts for approximately **45% of global pharmaceutical revenues**. CMS projects **national health expenditure of USD 6.017 trillion in 2026** (18.7% of GDP), after USD 5.662 trillion in 2025 and USD 5.279 trillion in 2024. Personal health care — hospital, physician and retail drug spend inside NHE — is projected at **USD 5.152 trillion in 2026**.

SERIES	2024	2025	2026
CMS NHE (USD trillions)	5.279 · 18.0% GDP	5.662 · 18.4% GDP	6.017 · 18.7% GDP
CMS personal health care	4.510	4.857	5.152
Insured share of population	91.8%	91.7%	90.8%

SYSTEM ARCHITECTURE

Population ~340 million (Census vintage). GDP per capita USD 82,000 (IMF 2025). ~6,120 hospitals; ~920,000 beds (2.7 per 1,000) — AHA 2024. CMS 2026 enrollment: Medicare ~69.2M; Medicaid ~80.8M; employer-sponsored ~179.8M.

REGULATORS AND PAYERS

FDA CDER (drugs), CBER (biologics), CDRH (devices). CMS for Medicare/Medicaid and IRA MFPs. PBMs — OptumRx, CVS Caremark, Express Scripts/Cigna. GPOs — Vizient, Premier, HealthTrust (~80% of hospital purchasing).

NHE and insured share: CMS Office of the Actuary, NHE Projections 2025–34 (June 2026), as published in Health Affairs. Pharma/device chips: BioNixus USA market intelligence.

03 Payer architecture

There is no single national purchaser. Commercial outcomes depend on which clock the product sits on — Medicare Part B, Part D, Medicaid, employer-sponsored insurance, or hospital GPO. CMS projects the insured share to ease to **90.8% in 2026** as enhanced Marketplace subsidies expire and direct-purchase enrollment falls.

PAYER / CHANNEL	2026 ENROLLMENT (CMS)	WHAT IT CONTROLS FOR MANUFACTURERS
Employer-sponsored insurance	~179.8 million	PBM formulary, rebates, step therapy, prior authorisation
Medicaid	~80.8 million	State preferred-drug lists, supplemental rebates, managed-care formularies
Medicare	~69.2 million	Part D formularies, Part B ASP+6%, IRA Maximum Fair Prices
Direct purchase / Marketplace	~34.5 million	Issuer formularies; enrollment down ~3.7 million vs 2025 on subsidy expiry

WHY THIS MATTERS FOR LAUNCH MODELS

A product with high Medicare mix must model IRA negotiation eligibility (small-molecule vs biologic exclusivity clocks) separately from commercial PBM net price. A hospital-administered biologic must model GPO and IDN value-analysis in parallel with CMS coverage. BioNexus does not collapse these into one “US access date.”

CMS projects Medicare spending to be the fastest-growing major funding source over 2025–34 (7.7% average annual). Retail prescription drugs are the fastest major spending *category* over the same window (5.7%).

Enrollment and growth rates: CMS NHE Projections 2025–34 presentation (24 June 2026). Channel implications: BioNexus access framework.

04 US pharmaceutical market

BioNexus sizes the US pharmaceutical market at **USD 590–640 billion in 2026** (hero midpoint **~\$615B**). This is a total-market band covering retail and physician-administered products — wider than CMS retail-prescription-drug spend inside NHE. The United States remains the world's largest national medicines market by value.

AUTHORISATION IS NOT ACCESS

FDA approval grants market authorisation only. Commercial access still requires PBM formulary placement, CMS coverage for Medicare, GPO contracts for hospital products, and — for selected high-spend Medicare drugs — IRA Maximum Fair Prices.

FDA PATHWAYS

NDA (21 CFR 314) and BLA (21 CFR 601), eCTD. PDUFA: 10 months standard, 6 months priority review. Breakthrough Therapy allows rolling review. Advisory Committee meetings are non-binding but highly influential.

LARGEST THERAPY AREAS BY COMMERCIAL SPEND

Oncology (largest and fastest-growing — immuno-oncology, targeted therapy, CAR-T). Immunology and biologics (TNF inhibitors at scale; IL-17/23 and JAK inhibitors; biosimilars accelerating since 2023). GLP-1 / obesity and diabetes (semaglutide and tirzepatide; potential **USD 50B+** category by 2030). Cardiovascular (novel heart-failure agents, PCSK9, novel anticoagulants). Rare disease and gene therapy (orphan designations; gene therapies at USD 1M+ list prices).

The Inflation Reduction Act is the most significant structural change to US pharmaceutical commercial models since the Medicare Modernization Act of 2003 — reshaping launch strategy, pricing architecture and international reference-pricing dynamics.

Pharma band: BioNexus USA market intelligence. Therapy ranking: published FAQ on /usa-healthcare-market-report.

05 US medical devices market

~\$180B

HERO CHIP 2026

\$175–200B

MDMA / ADVAMED BAND

FDA CDRH

DEVICE REGULATOR

BioNexus publishes **~\$180B** as the 2026 US devices midpoint, inside a **USD 175–200 billion** band (MDMA/AdvaMed). Hospital access is GPO-mediated: Vizient, Premier and HealthTrust control about **80%** of US hospital purchasing (3–9 months to contract).

DEVICE COVERAGE AFTER FDA CLEARANCE

- 1 **FDA CDRH clearance / approval** — risk class and 510(k), De Novo or PMA pathway.
- 2 **CMS coverage** — National Coverage Determination (~6 months) or automatic Part B/D coverage. TCET (Transitional Coverage for Emerging Technology) for qualifying devices.
- 3 **GPO contract** — Vizient, Premier, HealthTrust. Critical for hospital drugs and devices.
- 4 **IDN / hospital value-analysis committee** — parallel to GPO; determines on-shelf conversion.

Capital-equipment, consumables and IVD all sit in this commercial stack. ICER assessments can influence payer decisions but have **no statutory authority**.

Device band: BioNexus USA market intelligence (MDMA/AdvaMed). Access steps: same KPI registration path. Related page: /usa-medical-devices-market-report.

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Therapy landscape

Therapy	2026 Size	Growth	What moves the market
Oncology	USD 145–165B	11% CAGR	FDA Oncology Center of Excellence; 660+ oncology approvals 2017–2023; PD-1/PD-L1 USD 35B+
Immunology & biologics	USD 90–100B	12% CAGR	Adalimumab biosimilar wave from 2023; IL-17/23; JAK inhibitors
Cardiovascular	USD 75–85B	7% CAGR	TAVI, PCSK9, SGLT-2 HFrEF/CKD indications
Diabetes & metabolic	USD 60–70B	15% CAGR	Tirzepatide / semaglutide; IRA insulin cap USD 35/month Medicare
Rare disease	USD 35–40B	14% CAGR	7,000+ rare diseases; FDA Rare Disease Innovation Hub; gene therapy

EPIDEMIOLOGY (CITED SOURCES)

Condition	Published Stat	Source
Cancer	~2.0 million new diagnoses/year; breast, lung, colorectal, prostate most prevalent	ACS Cancer Facts & Figures 2024
Cardiovascular disease	~800,000 myocardial infarctions/year; #1 cause of death	AHA Heart Disease and Stroke Statistics 2024
Obesity	41.9% of adults obese — primary GLP-1/GIP demand driver	CDC NHANES 2023

Therapy sizes: BioNexus USA market-intelligence panel. Epidemiology: ACS / AHA / CDC as named. Bands are planning ranges, not SKU forecasts.

07 FDA, CMS and PBM sequence

10–24 mo REGULATORY APPROVAL	6–18 mo PAYER LISTING	19–51 mo LAUNCH TO ACCESS
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Breakthrough / priority review can compress the total clock to about **12–18 months**. There is no single national HTA body — payer-by-payer formulary negotiation is the primary access mechanism.

- 1 **Pre-NDA/BLA Type B meeting** — FDA; 90 days post-request. Align data; rolling review if breakthrough.
- 2 **NDA/BLA submission** — CDER or CBER. Day 0. eCTD.
- 3 **FDA technical review** — PDUFA 10 months standard / 6 months priority.
- 4 **Advisory Committee (if warranted)** — ~8–9 months into review. Public PI discussion.
- 5 **FDA approval letter** — PDUFA target action date.
- 6 **CMS coverage** — NCD ~6 months, or automatic Part B/D. Medicare Part B physician-administered infusions use ASP + 6%.
- 7 **Commercial PBM formulary** — Caremark, Express Scripts, OptumRx. 6–18 months. Step therapy and prior authorisation are common.
- 8 **GPO contract** — Vizient, Premier, HealthTrust. 3–9 months.

Registration steps and accessTimeline: BioNexus USA market intelligence on /usa-healthcare-market-report.

08 IRA, PBMs and formulary access

INFLATION REDUCTION ACT (2022) — THREE STRUCTURAL CHANGES

- 1 **Medicare drug price negotiation** — CMS negotiates Maximum Fair Prices for the highest-spend Medicare drugs: **10 drugs with MFPs in 2026**, expanding to **15 in 2027** and **20 per year** thereafter. First-cohort MFPs took effect January 2026. Published list-to-MFP reductions vary by product (commonly cited in a wide band; do not treat 60–80% as a universal cut).
- 2 **Inflation rebates** — manufacturers rebate CMS if price increases exceed CPI.
- 3 **Part D out-of-pocket cap** — beneficiary OOP capped at **USD 2,000 per year from 2025**.

IRA exposure is highest in cardiovascular, diabetes and oncology products with significant Medicare mix. Small-molecule vs biologic exclusivity clocks change when a product can be selected.

WHO ACTUALLY PLACES THE PRODUCT

ACTOR	ROLE
PBMs	OptumRx, CVS Caremark, Express Scripts/Cigna — formulary and rebate negotiations for most commercial and Part D plans
CMS	Medicare Part D coverage; Part B ASP+6%; IRA MFPs; NCD for selected products
GPOs	Premier, Vizient, HealthTrust — hospital supply-chain pricing
ICER	Influences payer decisions; no statutory authority

FDA approval does not guarantee commercial access. Model PBM, CMS and GPO clocks as separate workstreams — not a single national listing date.

IRA mechanics: statute and CMS negotiation timeline. Discount language: avoid a single average; product-level MFPs are public on CMS.

09 Hospitals and named centres

Mayo Clinic Rochester — 1,265 beds; #1 US News & World Report 2024; multi-specialty.

Cleveland Clinic — 1,400 beds; cardiology #1 in USA (US News); Heart & Vascular Institute.

Johns Hopkins Hospital Baltimore — 1,090 beds; oncology, neurology, transplant; Sidney Kimmel Comprehensive Cancer Center.

Massachusetts General Hospital — 1,000 beds; #3 US News; oncology, neurology, transplant.

UCSF Health — 800 beds; oncology, neurology, transplant — West Coast reference.

Memorial Sloan Kettering Cancer Center — 514 beds; #1 US oncology centre; precision oncology, CAR-T.

CAPACITY

~6,120 hospitals; ~920,000 beds (2.7 per 1,000). These six centres are the published account map, not an exhaustive IDN census. Community IDNs and regional systems still concentrate a large share of treated patients.

HOW BIONIXUS USES THE US MAP

The live hub at [/healthcare-market-research/united-states](#) is the service page. The report at [/usa-healthcare-market-report](#) is the sizing and access briefing. Commissioned work adds PBM-specific net-price, IRA cohort exposure and hospital-level consumption — not reproduced here.

Hospital list and bed counts: BioNixus USA market intelligence. AHA 2024 for national hospital census.

10 What BioNexus researches in the USA

This briefing is secondary intelligence. Sponsors commission BioNexus when the decision is primary: who will prescribe, who will cover, and which account actually converts.

MODULE	TYPICAL QUESTION	AUDIENCE
Physician U&A / demand	Share, line of therapy, switching, message fit	Specialists and high-volume community prescribers
KOL / AMC mapping	Who influences protocol and trial referral	NCI centres and regional KOLs
Payer / PBM advisory	Tier, PA, step, rebate pressure, IRA exposure	Medical directors and ex-PBM advisors
Hospital / IDN access	P&T, GPO, value-analysis, on-shelf conversion	Pharmacy and service-line leads
Patient / caregiver	Journey friction, OOP, persistence	HIPAA-aware, IRB-reviewed designs

US modules can run with comparable cells in the UK, EU5, Brazil, Saudi Arabia and the UAE on harmonised instruments. Minimum engagement **USD 20,000**. Typical proposal: 48 hours.

IRB/OHRP (45 CFR 46) and HIPAA/HITECH apply to US patient and many HCP programmes. BioNexus scopes ethics and privacy before fieldwork starts — they are not an afterthought on the critical path.

Service page: </healthcare-market-research-usa> · Hub: </healthcare-market-research/united-states>

11 Methodology and sources

Macro healthcare spending in this edition uses **CMS Office of the Actuary, National Health Expenditure Projections 2025–34** (forecast summary and Health Affairs publication, June 2026). Edition date: **5 September 2026**.

RECONCILIATION RULES

Headline healthcare chip is **CMS NHE 2026 ≈ USD 6.0 trillion** (published 6.017). Earlier BioNexus site copy used a USD 4.4–4.6T / ~\$4.5T band and a USD 5.0–5.2T KPI — those were superseded by the June 2026 CMS vintage and are not restated as current. Pharma remains a BioNexus band (USD 590–640B, midpoint ~\$615B). Devices remain USD 175–200B (MDMA/AdvaMed), midpoint ~\$180B.

CITED OFFICIAL AND NAMED SOURCES

CMS NHE Projections 2025–34 — NHE, PHC, GDP share, insured share, enrollment. US Census Bureau — population vintage. IMF 2025 — GDP per capita. AHA 2024 — hospital census. ACS 2024, AHA 2024, CDC NHANES 2023 — epidemiology. FDA PDUFA / 21 CFR — registration clock. IRA statute and CMS negotiation timeline — access.

PRIMARY CONSTRUCTION (FULL PROGRAMMES — NOT THIS PDF)

PBM formulary audits, hospital consumption analogues, GPO bid tracking, IRA cohort modelling, bilingual MENA fieldwork for US companies entering GCC. This briefing does not reproduce those microdata tables.

Gated briefing. Therapy SKU shares, rebate guarantees and hospital-level consumption remain in commissioned BioNexus work. Nothing herein is legal, medical, pricing or investment advice.

cms.gov NHE projections · healthaffairs.org 10.1377/hlthaff.2026.00642 · bionexus.com/usa-healthcare-market-report

NEXT STEP

Primary research, not a spreadsheet export.

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